

therapeutic potential of psychedelics in the EU and beyond.

We declare no competing interests. The views expressed in this Comment are the personal views of the authors and should not be understood or quoted as being made on behalf of or reflecting the position of the regulatory agency or agencies or organisations with which the authors are employed or affiliated.

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Why mental health should be embedded across climate and health discussions at COP29



Climate change is profoundly affecting mental health and psychosocial wellbeing by increasing the risk for new mental health conditions and making people who are living with pre-existing and severe mental health conditions more vulnerable.¹ Communities least accountable for climate change bear the brunt of these effects. Meanwhile, climate mitigation and adaptation actions can yield mental health co-benefits that are currently largely unaccounted for in climate policy.² Prioritising investments in mental health should be a core strategy in equitably addressing climate change across adaptation, mitigation, loss and damage, and just transition discussions, especially when considering the rights to wellbeing of future generations.³

The UN Climate Change Conference (or Conference of Parties; COP) is the supreme decision-making body of the UN Convention on Climate Change and the world's only multilateral decision-making forum on climate change, with almost every country represented in its membership. This year, COP29 will take place in Baku (Azerbaijan), where the effects of climate change on mental health are all too clear.

In the northern mountainous regions, farmers and herders are being forced to move away from their homes and sell their livestock as their livelihoods are disrupted by unpredictable rain patterns leading to floods and droughts. These changes have negatively affected mental health and psychosocial wellbeing by disrupting sleep, causing distress, and by contributing to psychosomatic symptoms and constant financial worries.⁴

Despite growing evidence on how mental health is affected by climate change, mental health has only recently had any space for discussion at COP. The first side event on mental health took place at COP26 in Glasgow (2021). The turning point for anchoring health within the climate negotiations was at COP28 in Dubai (2023), with the first ever Health Day. More than 150 countries have endorsed the COP28 UAE Declaration on Climate and Health, which includes an explicit commitment to including mental health as part of a comprehensive response to address the health effects of climate change.⁵

At COP29 in Baku, Azerbaijan, this progress should be built on with four strategic entry points (see panel

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for information on how to contribute to COP29 discussions in practice). The first of the strategic points is that at COP29, countries will discuss new climate financial targets for channelling greater funds towards climate action in developing countries (ie, the New Collective Quantified Goal; NCQG). These targets will replace the previous commitment of US\$100 billion annually from developed countries which falls far short of growing needs. Ambitious targets for accessible, grant-based, and flexible finance, based on the common but differentiated responsibilities principle, need to be agreed as part of the NCQG, because mental health cannot be safeguarded if climate action is not appropriately financed. Additionally, climate finance should be optimised for health, with strategic allocation of finance to projects that offer high returns on investment due to improved physical and mental health outcomes and associated economic savings. Furthermore, funding should be earmarked for mental health as part of existing financing commitments to climate and health (eg, \$1 billion announced at COP28) and as part of any future follow-up funding announced at COP29 (eg, as part of the Health Impact Investment Platform).

Second, a Multisectoral Actions Pathways Declaration for Resilient and Healthy Cities will be discussed as part of the high-level meeting on Climate and Urbanisation at COP29. As most of the world's population lives in cities, urban living can both protect mental health

(eg, via green space access and active modes of transport) and harm it (eg, via high exposure to air pollution). Multiple mental health co-benefits can come from low-carbon and climate-resilient cities. Mental health should be explicitly and meaningfully integrated in the implementation of the forthcoming Declaration.

Third, following Health Day at COP28, health-related discussions will be held on Nov 18, 2024, as part of Human Development, Children and Youth, Health, and Education Day. Mental health should be meaningfully integrated throughout Human Development Day and connected initiatives (eg, the Baku Initiative on Human Development for Climate Resilience and the Baku COP Presidency Continuity Coalition for Climate and Health), with action-oriented and tangible outcomes. In particular, the mental health of children and youth and their meaningful representation and engagement should be ensured at all levels of decision making, given their unique susceptibilities and the potential for considerable effect via early intervention.

Fourth, at COP29, governments will advance towards a set of adaptation indicators to be adopted at COP30 for the UAE Framework for Global Climate Resilience, a framework that positions adaptation as a top priority for all nations. Selected indicators should track information that is crucial for both physical and mental health.

Mental health has been increasingly recognised within global policy discussions on climate and health,⁸ from the UAE COP28 Declaration on Climate and Health to the 77th World Health Assembly's climate change and health resolution.⁹ However, there is still a long road ahead. Only 3% of countries' pledges to climate action (ie, Nationally Determined Contributions) mention mental health, whereas 28% cover vector-borne diseases.¹⁰ We hope that COP29 will effectively sustain the momentum built at COP28 and lay the groundwork for integrating mental health into the new round of Nationally Determined Contributions due in 2025, in the lead up to COP30 in Belem do Pará, Brazil.

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Panel: Practical contributions to COP29 discussions

- Join the Global Mental Health Action Network's (GMHAN) Environment Working Group, the Global Climate and Health Alliance, and the Connecting Climate Minds Hub to connect with other advocates and experts from across the world
- Advocates: engage with policy makers to raise awareness of the climate change and mental health intersection and call for better action using available resources—eg, GMHAN's advocacy briefs⁶ available in English, Spanish, French, Portuguese, and Azerbaijani
- Health-care professionals: follow WHO recommendations⁷ on how to communicate effectively the health threats from climate change and the need for climate action to patients and communities
- Researchers: use the Connecting Climate Minds research and action agendas to inform research
- People with lived experience: highlight how climate change has affected mental health and what needs to change

For the **Global Mental Health Action Network Environment Working Group** see <https://gmhan.org/environment-working-group>

For the **Global Climate and Health Alliance** see <https://climateandhealthalliance.org/>

For the **Connecting Climate Minds Hub** see <https://hub.connectingclimateminds.org>

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Global action on problematic usage of the internet: announcing a *Lancet Psychiatry* Commission



Digital technologies and societal problems are increasingly interconnected, and this nexus has important implications for mental health globally.^{1,2} Concerns exist about problematic usage of the internet (PUI), digital technology-related disorders, and their public health and societal costs.³ One 2020 meta-analysis found that around 7% of the world's population show signs of internet addiction, and prevalence estimates have shown an increase over time, especially among young people.⁴ Specific types of PUI, such as problematic smartphone use, might have an even higher prevalence.⁵ Increased awareness of the association between PUI and mental disorders in general and the potential negative effects of social media use in particular has led to calls for more action to protect individuals, particularly children and youth, from online harm.^{6,7}

Specific types of PUI have been recognised as mental and behavioural disorders in the ICD-11. These include online forms of gambling disorder and gaming disorder, which are classified primarily as disorders due to addictive behaviours and secondarily as impulse control disorders, and compulsive sexual behaviour disorder, which is classified as an impulse

control disorder and often expressed online. Other digital technology-related behaviours might be considered as other specified disorders due to addictive behaviours, including compulsive buying or shopping behaviours and problematic usage of social media.⁸ Important advances have been made in developing clinical measures for these conditions, and research is accumulating on the overlapping and distinctive psychobiology, psychotherapy, pharmacotherapy, and neuromodulation related to specific types of PUI.³

Despite important efforts, much more work is needed from multiple stakeholders in clinical, public health, governmental, and other sectors.⁹ Key questions for society to address include the biological, psychological, social, and commercial risk and resilience factors that lead to or mitigate against PUI, the individual and public health consequences of PUI, and the nature and outcomes of health systems and services for individuals with PUI. A key area is the relevance to PUI of regulatory frameworks related to substances, gambling, and harmful consumer products: aspects of these regulatory frameworks might be applicable to PUI, but several unique aspects of PUI seem to require specific approaches. Particular concerns are the ready access of

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